

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HPAA Discrepancy		☒
Prior Malignancy history		☒
Dual/Synchronous Primary Notes		☒
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	RB	
Date Reviewed	12/21/11	
	1/7/12	

UUID: F1399697-D1A7-4B59-ACF2-1DDE8C6733AF
TCGA-EL-A3N2-01A-PR Redacted



Surgical Pathology Report

Department of Pathology
Tel:

DOB:
Physician:

Sex: F

Collected:

Accession:

Received:

Case type: Surgical Case

DIAGNOSIS

(A) LEFT LATERAL NECK DISSECTION:

METASTATIC PAPILLARY THYROID CARCINOMA IN 2 OF 17 LYMPH NODES (2/17)

(B) LEFT LARYNGEAL NERVE, QUERY TUMOR:

PAPILLARY THYROID CARCINOMA IN SOFT TISSUE.

1C0-0-3

carcinoma, papillary, thyroid 8360/3
Site: thyroid, NOS C73.9

(C) QUESTION "LEFT SUPERIOR PARATHYROID":

Fibroadipose tissue, negative for tumor.

(D) TOTAL THYROID AND BILATERAL LEVEL VI:

PAPILLARY THYROID CARCINOMA, CONVENTIONAL TYPE

Location: Left lobe

Multi-focal: No

Size = 3.5 cm

Extrathyroidal extension: Present, extending into fibroadipose tissue and abutting skeletal muscle

Resection Margins: Negative

Lymph nodes:

METASTATIC PAPILLARY THYROID CARCINOMA IN 3 OF 10 LYMPH NODES (3/10)

Location: Bilateral, level VI

Extracapsular extension: Present

Parathyroid, negative for tumor.

Thymus parenchyma, negative for tumor.

1/7/12

GROSS DESCRIPTION

(A) LEFT LATERAL NECK DISSECTION - An unoriented fragment of tan-yellow adipose tissue (11.0 x 5.0 x 1.0 cm).

Two possibly grossly positive lymph nodes are present, 1.5 and 2.5 cm. The larger positive lymph node has a cystic component. Remainder of the specimen consists of multiple possible lymph nodes ranging from 0.3 to 1.3 cm.

SECTION CODE: A1, largest positive lymph node entirely submitted; A2, second possibly positive lymph node entirely submitted; A3, A4, one possible lymph node in each bisected; A5, four possible lymph nodes; A6-A8, three possible lymph nodes in each.

(B) LEFT LARYNGEAL NERVE, QUERY TUMOR - A single fragment of red-brown tissue (0.4 x 0.3 x 0.1 cm). Entirely submitted in B1 for frozen section diagnosis.

*PAPILLARY THYROID CARCINOMA PRESENT IN NEUROVASCULAR BUNDLE.

Surgical Pathology Report

Department of Pathology,
T

DOB: :
Physician:

Sex: F

Collected:
Received:

Pathologist:
Case type: Surgical Case

Accession:

(C) QUESTION LEFT SUPERIOR PARATHYROID – A tiny soft tissue (0.1 x 0.1 x 0.1 cm, weighing 0.005 grams) is submitted entirely in C1 for frozen diagnosis.

*FS/DX: BENIGN FIBROADIPOSE TISSUE.

(D) TOTAL THYROIDECTOMY AND BILATERAL LEVEL VI - A (4.5 x 3.5 x 1.5 cm) left thyroid lobe with attached (3.5 x 2.5 x 0.2 cm) fragment of adipose tissue and a detached right thyroid lobe (4.0 x 2.0 x 1.0 cm). The entire specimen weighs 0.02 kg.

A yellow to brown mass entirely replaces the left lobe. This mass extends to involve the external surface where there is a 2.0cm are of papillary excrescences. The cut surface of this lesion is also diffusely firm with no areas of hemorrhage or necrosis.

The right lobe consists of red-brown unremarkable thyroid tissue. The attached adipose tissue is sectioned to reveal multiple lymph nodes ranging from 0.3 to 0.7 cm with a 0.7 cm lymph node grossly positive.

SECTION CODE: D1-D7, lesion in left lobe with cassettes 1-5 containing papillary external surface extension; D8-D10, representative right lobe; D11, one grossly positive lymph node entirely submitted; D12, three possible lymph nodes; D13, four possible lymph nodes; D14-D16, remainder of adipose tissue entirely submitted.

CLINICAL HISTORY

Papillary Thyroid cancer

SNOMED CODES

T-B6000, M-80503, T-C4200, M-80506, "Some tests reported here may have been developed and performance characteristics determined by
These tests have not been specifically cleared or approved by the U.S. Food and Drug Administration."

Entire report and diagnosis completed by: -----

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File under: Pathology		